

# Sodium reduction

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## Key facts

- Almost all populations are consuming too much sodium.
- The global mean intake of adults was 4278 mg/day sodium (equivalent to 11 g/day salt) in 2021 (1). This is more than double the World Health Organization recommendation for adults of less than 2000 mg/day sodium (equivalent to < 5 g/day salt, or approximately one teaspoon).
- The primary health effect associated with diets high in sodium is raised blood pressure which increases the risk of cardiovascular diseases, gastric cancer, obesity, osteoporosis, Meniere's disease, and kidney disease.
- An estimated 1.7 million deaths each year were associated with consuming too much sodium in 2023 (2).
- Reducing sodium intake is one of the most cost-effective measures to improve health and reduce the burden of noncommunicable diseases: for every US\$ 1 invested in scaling up sodium reduction interventions, there will be a return of at least US\$ 12.

## Overview

The [burden of unhealthy diets](#) constitutes a major public health and development challenge worldwide. Urgent action is required to modify the over-production and over-consumption of foods and beverages that do not have a healthy nutritional profile, mainly industry manufactured food. Of greatest concern is excess consumption of sodium, sugars and unhealthy fats, particularly *trans*-fatty acids (trans fats) and saturated fatty acids, and low consumption of whole grains, pulses, vegetables, and fruits. In many high-income countries, and increasingly in low- and middle-income countries, a significant proportion of sodium in the diet comes from processed foods.

An estimated 1.7 million deaths each year were associated with consuming too much sodium in 2023, a well-established cause of raised blood pressure and increased risk of cardiovascular disease (2).

Reducing sodium intake is one of the most cost-effective ways to improve health and reduce the burden of noncommunicable diseases, as it can avert many cardiovascular events and deaths at very low total programme costs. WHO recommends several sodium-related best buy policies as practical steps that countries can take to prevent cardiovascular disease and its associated costs. These include [lowering sodium content in foods](#); implementing [front-of-pack labelling](#); mass media campaigns; and [public food procurement and service](#).

## About sodium

- Sodium is an essential nutrient necessary for the maintenance of plasma volume, acid-base balance, transmission of nerve impulses and normal cell function.
- Sodium deficiency is extremely unlikely in healthy individuals.
- Excess sodium is linked to adverse health outcomes including increased blood pressure.
- The primary contributors to dietary sodium consumption depend on the cultural context and dietary habits of a population.
- Sodium and chloride are the main components of regular table salt, but sodium can be found in other forms and is also found naturally in a variety of foods such as milk, meat, and shellfish. It is often found in high amounts in processed foods such as breads, processed meat, and snack foods, as well as in condiments (e.g., soy and fish sauce).
- Sodium is also contained in sodium glutamate, which is used as a food ingredient and additive in many parts of the world.

## Recommendations for reducing sodium

- For adults, WHO recommends less than 2000 mg/day of sodium (equivalent to less than 5 g/day salt), or just under a teaspoon.
- For children aged 2–15 years, WHO recommends adjusting the adult dose downward based on their energy requirements. This recommendation for children does not address the period of exclusive breastfeeding (0–6 months) or complementary feeding with continued breastfeeding (6–24 months).
- If choosing to use table salt, WHO suggests replacing regular table salt with [lower-sodium salt substitutes](#) that contain potassium.
- All salt that is consumed should be [iodized \(fortified with iodine\)](#), which is essential for healthy brain development in the foetus and young child and optimizing mental function in general.

# Public health interventions for sodium reduction

WHO has identified a set of evidence-based [best buy interventions to tackle noncommunicable diseases](#). If undertaken immediately, these interventions are expected to accelerate results in terms of lives saved, healthy years of life gained, cases of disease prevented, and costs avoided.

There are 4 best buys for sodium reduction:

- reformulate food products to contain less sodium and set target levels for the amount of sodium in foods and meals;
- establish a supportive environment in public institutions such as hospitals, schools, workplaces and nursing homes to provide lower sodium options;
- implement front-of-pack labelling; and
- apply behaviour change communication and mass media campaigns.

The development, implementation, monitoring, and evaluation of sodium reduction policies should be government-led and safeguarded against possible conflicts of interest.

WHO has developed a [Sodium Country Score Card](#) to monitor countries' progress in making national commitments and in taking a multifaceted approach to implement policies to reduce sodium intake.

## How can you reduce your sodium intake?

- eat mostly fresh, minimally processed foods
- remove the saltshaker/container from the table
- cook with little or no added sodium/salt
- use herbs and spices to flavour food, rather than salt
- replace regular table salt with lower-sodium salt substitutes that contain potassium
- limit the use of commercial sauces, dressings and instant products
- limit the consumption of processed foods
- choose foods with lower-sodium content, or low-sodium products.

## Sodium and salt converter

Convert the sodium listed on food products into grams of salt and vice versa.

- 1 gram of salt is equivalent to 400 mg of sodium
- 5 grams of salt is equivalent to 2000 mg of sodium

## WHO response

[WHO guidelines on sodium](#) provide thresholds for healthy intake and outline measures for improving diets to prevent noncommunicable diseases (NCDs) in adults and children.

The [Global strategy on diet, physical activity, and health](#) was adopted in 2004 by the World Health Assembly. It calls on governments, WHO, international partners, the private sector and civil society to act at global, regional, and local levels to support healthy diets and physical activity.

In 2010, the World Health Assembly endorsed a [set of recommendations on the marketing of foods and non-alcoholic beverages to children](#). These recommendations guide countries in designing new policies and strengthening existing ones to reduce the impact of unhealthy food marketing on children. The WHO Regional Offices have developed nutrient profile models that countries can use as a tool to implement these marketing recommendations.

In 2011, world leaders committed to reducing people's exposure to unhealthy diets. The commitment was made through a [Political Declaration of the High-level Meeting of the United Nations General Assembly on the Prevention and Control of NCDs](#).

In 2012, the World Health Assembly adopted [6 global nutrition targets](#), including the reduction of stunting, wasting and overweight in children, the improvement of breastfeeding and the reduction of anaemia and low birth weight.

In 2013, the World Health Assembly agreed on [9 global voluntary targets](#) for the prevention and control of NCDs, which include a halt to the rise in diabetes and obesity and a 30% relative reduction in the intake of salt by 2025. [The Global action plan for the prevention and control of noncommunicable diseases 2013–2020](#) gives guidance and a menu of policy options for Member States, WHO and other UN agencies to achieve these targets. The timeline for achieving the targets was extended to 2030 by a [World Health Assembly decision in 2019](#) to ensure its alignment with the 2030 Agenda for Sustainable Development.

In 2023, the first [Global report on sodium intake reduction](#) was published based on the [Sodium Country Score Card](#). The report aims to monitor progress and identify areas for action in implementing sodium reduction policies and other measures within Member States and across WHO regions and World Bank income groups.

## References

1. [Noncommunicable Diseases Data Portal](#). Geneva: World Health Organization; 2025.
2. [GBD results \[online database\]](#). Seattle: University of Washington; 2025